From a working product to a paying partner

MediChain is built and live. The business question is who pays first, what it costs to serve them, and what could go wrong.

Customers, in order

Segment	Why they buy	First offer
Private EMS operators	Responders need allergies and medicines on scene; they handle few records, so they are quick to adopt	Emergency summary and handover, per vehicle or per crew
Private clinics and GP groups	One shared record across their sites; patients who can see their own care	Staff portal and patient app, per facility
Hospital groups	Emergency department intake and handover; accountability for access	Integration pilot in one emergency department
Public facilities	National policy goals; after POPIA authorisation and procurement	Through partners and programmes

Revenue model

- **Subscription per facility or vehicle**, priced by active staff, with onboarding and support. Free for patients, always.
- **Implementation fees** for integration with an existing hospital system (FHIR endpoints already exist).
- Prices are hypotheses to set with the first partner; we will not publish numbers we have not tested.

Cost to serve

Item	Today	At pilot scale
Hosting (AWS Cape Town)	One t3.small instance, S3; budget alarm at USD 60 a month	A larger instance and a managed database; cost grows with facilities, not with patients
Distribution	Web apps: no app-store fees or review delays	The same, plus an Android app for NFC card emulation
Support	Founder	A clinical lead and a support engineer

Competitive position

Alternative	Strength	MediChain's angle
Paper folders	Familiar; no infrastructure	Available at the point of care; every access recorded
Hospital record systems (OpenMRS, Bahmni)	Mature clinical operations	Patient-held emergency summary and patient-visible access; can sit beside them
Emergency apps (iER)	Panic alerts, self-reported details	Clinician-maintained data, approved devices, reads shown to the patient

Referral platforms (Vula)	Clinician-to-specialist referrals	Complementary: the patient's own record and emergency access

Comparisons describe focus, not shortcomings.

Go-to-market

Phase	Milestone
1. Pilot (3 months)	One EMS or clinic partner; synthetic training, then an approved pilot of the emergency summary and handover
2. Proof (6 months)	Published pilot measures: seconds to critical information, handover completeness, patient understanding
3. Expansion	Paying facilities in Gauteng and the Western Cape; an Android app; integrations

Risks and how we handle them

Risk	Response
Regulatory approval takes time	Start with fictional and consented pilot data; Information Officer and POPIA prior authorisation are tracked as release gates
Clinicians will not adopt another system	Begin where the pain is sharpest (the emergency summary), keep each workflow to a few screens, measure adoption per shift
Patients lose phones or cards	A card number works by hand; the record is on the server, not the device
Single founder	First hires are a clinical lead and a second engineer; the code, tests and runbooks are written so others can take over
Connectivity and load-shedding	An offline QR summary; light web apps

Team and funding

Keorapetswe Kgoatlha, founder and developer, designed and built MediChain end to end (2nd place, Rust Africa Hackathon 2026). Company: Lukau Invasion (Pty) Ltd, 2026/600804/07. Funding sought from grants (for example NYDA and youth-entrepreneurship funds) and pilot revenue, for the first two hires and the regulatory work.

The ask

A pilot partner, and introductions to funders who back South African health technology.